Run the hospital claims desk first

Build a hospital-side insurance claims operator: a company that takes over a private hospital's cashless-claim desk and is paid for claims settled in full and on time, starting with a contingency-priced audit and recovery of recent deductions and denials. Of the six candidates it is the only one that combines a large, measured flow of money (about ₹62,500 crore a year of cashless payouts, with roughly 22% of claimed value disallowed or repudiated in FY24), open and visible conflict between buyers and payers, a regulator pushing speed, and no rule found that reserves the work to a licensed profession or a regulated entity. It scores 33 out of 45 on the nine criteria below, six points ahead of the next two candidates, and it matches or beats the tax and insurer-side options on every single criterion. The runner-up is a GST notice-to-resolution desk for mid-market and large taxpayers, delivered with employed chartered accountants. The recommendation rests on a thin fact base in exactly the places that matter commercially: no published Indian price for any hospital-side claims vendor, no audited data on hospital deduction rates or receivable days, no sales-cycle data for any of the six sectors, and no official NHCX adoption figures after mid-2024. The illustrative model in this report produces about ₹12 lakh of annual revenue per mid-sized hospital, which makes the domestic business modest on its own; the venture-scale case depends on expansion into receivables financing, government schemes and US revenue-cycle work. The first 30-60 days should therefore be spent testing price, recoverability and payer-portal access with real hospitals before any platform is built.

A note on evidence. Most figures here come from press reports of regulator data, vendor blogs and search-result summaries, and the underlying primary documents were mostly not opened. Rupee-to-dollar conversions use roughly ₹85 = US$1 and are approximate. Where a figure is unverified, conflicting or a vendor claim, the text says so. Numbers in the unit-economics model are assumptions unless a source is linked.

Hospital claims wins six-way comparison by six points

How the six candidates score

Each candidate is scored 1 to 5 on nine criteria, where 5 is always best for the founder (so a 5 on competition means an open field, a 5 on liability means low exposure, and a 5 on exception-rate risk means few cases needing human handling). The scores are my judgement applied to the research notes, not measured quantities, and the totals are unweighted.

Criterion	Hospital claims desk (healthcare)	Insurer claims investigation	Legal operations	Tax notices and reconciliation	Export compliance	KYC / onboarding ops
Money at stake	4	4	4	4	3	2
Pain and urgency	5	3	3	4	3	3
Regulatory tailwind	4	4	2	4	4	2
Buyer access and sales cycle	3	1	3	3	2	2

Criterion	Hospital claims desk (healthcare)	Insurer claims investigation	Legal operations	Tax notices and reconciliation	Export compliance	KYC / onboarding ops
Integration feasibility	3	3	2	3	2	4
Competition (5 = open)	3	2	3	2	4	1
Liability (5 = low)	4	2	2	2	3	2
Fit for outcome pricing	4	4	2	3	3	2
Exception-rate risk (5 = low)	3	3	1	2	3	3
Total (of 45)	**33**	**26**	**22**	**27**	**27**	**21**

The ranking is not sensitive to weighting. The hospital claims desk scores at least as high as tax and insurer-side investigation on every criterion, so no weighting can put either ahead. Export compliance beats it only on competition, and that criterion would need more than seven times the weight of the others to close the gap. Buyer-access scores are the least well supported in the table, because the notes found no sourced sales-cycle length for hospitals, insurers, lenders, tax teams or exporters.

Healthcare: hospital revenue cycle and claim rejections (33)

Regulated health insurance paid about **₹94,248 crore on roughly 3.26 crore claims in FY25**, with about 66% of the amount going cashless, which means through a hospital's insurance desk (India Med Today). That puts the cashless flow at about ₹62,500 crore (roughly US$7.4bn), which is the notes' arithmetic and not a published figure. In FY24, by value, **₹15,100 crore was disallowed and ₹10,937 crore repudiated out of about ₹1.17 lakh crore claimed**, or about 22% (Outlook Business). Much of that is legitimately non-payable (sub-limits, exclusions, consumables) and it includes policyholders' reimbursement claims, so the share a hospital-side vendor could recover is a fraction that no source quantifies. The FY25 equivalent appears in one low-authority source only and is unverified.

Pain is the strongest of any candidate and it is public. The hospital association AHPI advised members to suspend cashless service for Bajaj Allianz from 1 September 2025 over deductions, delays and slow approvals, and warned Star Health the same month (Outlook Money; Business Today). A NATHEALTH paper describes settlement delays of about 45 days as a common complaint (NATHEALTH). Vendor estimates, which are unaudited, put losses at 8-14% of claim value for a 100-200 bed hospital (Docxtract). The regulator's May 2024 master circular requires insurers to decide cashless authorisation within one hour and final discharge authorisation within three hours (ThePrint); those clocks bind insurers, but they shorten the window in which a hospital must answer queries, which favours an always-on operator.

The buyer universe is workable in size: Medi Assist reports 12,000+ hospitals actively submitting claims each month and IHX says it connects 30,000+ (Medi Assist; YourStory). Competition exists but is thinly capitalised. Care.fi raised about US$8M in February 2026 and bundles its claims software with receivables financing (ImpactAlpha); ClaimBuddy raised US$5M (Outlook Business); Vitraya about US$4M (Inc42). The control

points belong to payer-side incumbents, IHX (which claims over 40% of cashless claims) and Medi Assist, who sit on the other side of the table and are adding AI themselves. The liability score is high because the hospital is filing its own claim: the notes found no professional-practice statute and no insurer or bank outsourcing rule that applies to a vendor working for the hospital. That is an absence of evidence and needs a legal check.

The score is held back by three things. Integration today means payer portals, email and spreadsheets, since the only official NHCX adoption figures date from mid-2024 (34 insurers and TPAs live, about 300 hospitals preparing) (India Insurtech Summit). Government-scheme dues are a payer-liquidity problem that software cannot fix. And there is no published willingness-to-pay data at all.

Insurance: claims investigation (26)

The money is well measured: BCG and Medi Assist put fraud, waste and abuse in health insurance at **₹8,000-10,000 crore a year**, or 8-10% of payouts (BCG). That is roughly ₹2,450-3,070 of leakage per claim by the notes' arithmetic, several times the ₹813-905 that Medi Assist earns per claim (a derived upper bound). IRDAI's fraud monitoring framework took effect on 1 April 2026 (JSA). The problem is the buyer. There are about 33 general and standalone health insurers and roughly 20 TPAs, so around 50 logos (Ditto), and the largest TPA already markets its own fraud engine and reports ₹390 crore of fraud prevented in FY25 (FilingReader). A handful of contracts would make or break the company. The 2017 IRDAI outsourcing regulations barred insurers from outsourcing claims decisions (TaxGuru); a newer framework reportedly loosens what counts as core, but the current instrument was not verified. Motor, life and crop claim counts, investigation costs and referral rates could not be sourced.

Legal operations (22)

The stock of work is enormous: 44.7 lakh pending cheque-bounce cases (Lokmat Times) and about 10.7 lakh motor-accident claims worth roughly ₹96,000 crore (Legacy IAS). Structure kills the score. Bar Council rules bar advocates from sharing fees or partnering with non-advocates, a rule still in force as of a May 2026 request to amend it (The Tax Corp), so a company cannot be the law firm. Cycles run to years (about six at motor-accident tribunals) (LiveLaw), which makes "tracked to result" pricing impractical, and e-filing remains uneven, which forces physical filing and local appearances. Legal market-size estimates differ by 17 times across three reports and are unusable. Almost all legal figures in the notes are from search summaries.

Taxation: notices, reconciliation and compliance (27)

Tax is the most automatable candidate on paper. The GST base is machine-readable: 1.67 crore active registrants and about 21.4 crore e-invoices in June 2026 alone (A2Z Taxcorp on the GSTN nine-year report). The dispute layer was unblocked when the GST Appellate Tribunal opened, with over 4 lakh first-appeal orders newly appealable and about 1.8 lakh first appeals pending (TaxGuru). Non-lawyers may represent taxpayers. Three things cap the score. Filing and reconciliation are crowded (Clear at ₹272 crore revenue with a ₹96 crore loss, Vyapar's acquisition of Suvit, 62 GST Suvidha Providers) (Entrackr), and the low-end price anchor for an AI notice reply is ₹299 (Product Hunt). Only a CA, advocate, employee or enrolled practitioner may sign and appear under section 116, so the company must employ professionals or sell into their firms (Patron Accounting). And the two numbers that would size the market, an official national count of GST notices and the value of input-tax-credit mismatches, were not found. No official API for notices, replies or appeals was evidenced either.

Exports: customs and trade documentation (27)

No funded Indian company was found doing end-to-end export compliance, which is why competition scores 4; absence in search results is weak evidence because small Indian startups are poorly indexed. Rule churn is real: RoDTEP incentive rates were halved and restored within a month in early 2026, and a new foreign-exchange regime for trade took effect on 1 October 2026 (Policy Circle; Lakshmikumaran & Sridharan). The weaknesses are price and plumbing. Brokers charge roughly ₹2,500-5,000 per export shipment (broker blogs that do not agree with each other) (Skydo), the active buyer base is about 1.73 lakh MSME exporters per a secondary source (M1NXT), and nobody publishes how much incentive money goes unclaimed. The API route into the customs system depends on a "Customs Suvidha Provider" accreditation that, as of a 2024 note, had no developer portal or sandbox, and no live providers were confirmed (TaxGuru).

Banking: KYC and onboarding operations (21)

Volume is vast, with 44.63 crore Aadhaar e-KYC transactions in March 2025 alone (Deccan Chronicle), but an automated check sells for roughly ₹3-18 by vendor estimates (upGrowth). After a decade the pure-play leaders each report only about ₹127-189 crore of revenue (Entrackr on IDfy; Inc42 on Digio). RBI's August 2025 amendment requires video KYC to be operated only by the regulated entity's own trained officials (HyperVerge), which caps what a third party can "complete", and the June 2025 amendment eased re-KYC towards self-declaration and business correspondents. The plausible gap, business KYC and money-laundering alert triage, was not sized by any source.

The runner-up and what would change the choice

The runner-up is a GST notice-to-resolution desk for taxpayers above roughly ₹100 crore turnover, staffed with employed CAs who sign and appear. It ties with export compliance on points; I rank it ahead because fees can scale with the disputed demand, value is concentrated (0.10% of GST taxpayers pay 50.29% of cash tax, per the GSTN report linked above), and the data sits on national portals, whereas the export product's revenue per shipment converges on a broker's fee.

Four findings would change the recommendation. If hospital interviews show that most deductions are contractually non-payable and unrecoverable, or that finance heads will not pay more than a token per-claim fee, the hospital thesis fails and tax becomes the pick. If payer portals or the IHX and Medi Assist rails prohibit or block a third party operating on a hospital's credentials, integration risk rises sharply. If an official notice count shows several lakh contestable GST notices a year and CFOs accept demand-linked fees, tax closes the gap. If customs accreditation is live and open to new entrants, exports rises. Founder background also matters more than a six-point gap: a founder with a CA network, a customs-broker licence or insurer relationships should weigh that against this ranking.

A cashless-claims operator paid on money recovered

Target customer and wedge

The first customers should be private multi-specialty hospitals and small regional chains with enough cashless insured volume to employ a dedicated insurance desk, in one or two cities. I suggest the 50-300 bed band as a working assumption; the notes contain no sourced distribution of hospitals by bed size, only a 2019 study indicating that most private hospitals have fewer than 50 beds (NIPFP). Hospitals whose revenue is dominated by PM-JAY or state schemes should wait, because their main problem is unpaid dues. Several

hundred private hospitals in Haryana suspended scheme services in August 2025 over about ₹490 crore owed (Business Standard), and PM-JAY claims run in a separate government system outside NHCX (Nirmitee).

One caution on scheme figures. The widely reported "₹1.21 lakh crore" of pending PM-JAY claims is almost certainly a tenfold press error. The rupee figure quoted in the same report, Rs 1,21,61,45,63,617 across 63.9 lakh claims, equals about ₹12,161 crore (Medical Dialogues). That correction is the research team's arithmetic and has not been published anywhere, so do not size anything off the larger number.

The wedge is a retrospective settlement audit sold on contingency. The hospital hands over its recent claim files, settlement letters and bank statements; the operator reconciles what was paid against what was claimed, identifies deductions and denials that can be contested, files the appeals and takes a share of what comes back. This needs no system integration, creates the baseline that outcome pricing later depends on, and attacks the task the notes describe as most manual: a single bank transfer may cover 40-80 claims netted with deductions (Patron Accounting). The recoverable window is short. One consultancy puts the denial-resolution window at 45-90 days (Ichelon Consulting), so the audit covers recent months, and the actual contractual windows must be checked per payer. Once trust and a baseline exist, the operator takes over the live desk from pre-authorisation to settlement.

Seven workflow stages mapped to product modules

Stage	Module	What it does	Human role
1. Collect documents	Intake hub	Pulls admission, policy, identity, clinical and billing documents from the hospital system, scans, email and messaging; builds a per-admission checklist by payer and procedure	Desk staff photograph or upload what the system cannot fetch
2. Extract	Claim file builder	Turns discharge summaries, investigation reports, bills and implant invoices into structured fields with a link back to the source line	Reviewer checks low-confidence fields only
3. Check against official rules	Payer rules engine	Tests the file against policy terms, sub-limits, room-rent caps, package rates, the hospital's tariff agreement with each payer, and the regulator's timelines	Rules team maintains the library and signs off changes
4. Find missing evidence	Gap finder	Lists what the payer will ask for before it asks, and drafts requests to the treating doctor or billing	Treating doctor supplies or signs clinical documents
5. Prepare the claim	Submission composer	Assembles pre-authorisation, enhancement, discharge and final-bill packets in each payer's format, with a justification note	Medical reviewer approves high-value and flagged cases
6. Submit or route with authorisation	Channel layer	Files through payer and TPA portals, email, and NHCX where live, under the hospital's written authority and a named approver; every action logged	Hospital approver releases, by rule or case by case
7. Track result and handle exceptions	Query, reconciliation and appeals desk	Answers payer queries against the clock, matches settlements to bank credits, classifies each deduction, files appeals and escalations	Appeals specialists handle contested and clinical disputes

Human-in-the-loop design

Every global company in the notes that sells a regulated outcome keeps people in the delivery loop. EvenUp has more than 100 in-house nurses, paralegals and lawyers reviewing output (Sacra), and the US medical-billing startup Altair Health states that its in-house billers handle the exceptions (Altair Health). The design here has three tiers. Routine claims that pass all rule checks go out after a hospital approver's release. Claims with low extraction confidence, high value or a rules conflict go to an operator-side reviewer. Clinical disputes and appeals go to a doctor or senior coder. Each human correction is captured as training and rules data, which is the learning loop Bessemer names as a route to software-like margins (Bessemer).

One rule is absolute: the system never creates, alters or infers clinical facts. It may point out that a document is missing; only the treating clinician may supply it. This matters commercially as well as ethically, because the regulator's fraud framework extends to insurers' vendors and partners and fraud propensity is highest in mid-value claims (Angel One). A vendor seen as engineering claims will be shut out by payers. No published exception rate exists for Indian health claims or for any AI-services company, so the share of claims needing each tier is unknown until pilots measure it.

Data and integration layer

Plan for the world as it is. Volume still runs on private rails and portals, so the channel layer must work through payer and TPA portals, email and the hospital's own billing export, with NHCX as an option. NHCX offers open, FHIR-based APIs through the national digital health sandbox, with the hospital identified by its Health Facility Registry ID, and hospitals receive an incentive of ₹500 per claim or 10% of claim amount, whichever is lower, for claims sent through it; both details rest on one vendor's blog and were not checked against official documentation (Nirmitee onboarding checklist; Nirmitee). Reports on adoption conflict: a December 2025 interview with the National Health Authority says all insurers are on NHCX but hospital participation is slow (ThePrint), and control of the exchange was contested between ministries in 2025 (Business Standard).

The durable asset is the data model: one record per admission linking policy, clinical documents, bill lines, every payer query, each deduction with its stated reason, and the bank credit that settled it. IRDAI told Parliament in July 2026 that it holds no data on why claims are denied (Insurance Business). A cross-hospital dataset of denial reasons by payer and procedure is therefore something nobody, including the regulator, currently has.

Pricing and an illustrative unit-economics model

Price in two parts: a low per-claim operating fee for running the desk, and a share of money recovered above an agreed baseline. US anchors are 5-9% of collections for traditional vendors and a 1-1.5% target for an AI-led roll-up (MedCity News). The hospital frames the pain as money withheld, and the closest competitor bundles financing, which suggests software fees alone have been hard to sell; that is inference, not evidence. No Indian price for any hospital-side vendor has been published.

The model below is for one mid-sized hospital. Every input is an assumption chosen for illustration unless a source is cited, and none should be quoted as a finding.

Input or result	Value	Status
Cashless claims per month	300	Assumption

Input or result	Value	Status
Average cashless inpatient claim	₹50,000	Assumption. The sourced FY25 average of ₹28,910 covers all claims including small outpatient ones (India Med Today)
Claimed value per month	₹1.5 crore	Calculated
Baseline leakage (deductions, denials, rework)	10% = ₹15 lakh a month	Assumption, inside the unaudited 8-14% vendor range (Docxtract)
Share of leakage the operator prevents or recovers	25% = ₹3.75 lakh a month	Assumption with no source; the single most important unknown
Operating fee	₹150 per claim = ₹45,000 a month	Assumption
Success fee	15% of recovered amount = ₹56,250 a month	Assumption
Revenue per hospital	**about ₹1.01 lakh a month, ₹12.2 lakh a year**	Calculated; about 0.7% of claimed value
Operations staff time	20 minutes per claim at ₹150 an hour = ₹50	Assumption; hourly rate follows the notes' illustration from a ₹3 lakh salary, consistent with claims-processor pay data (Payscale)
Specialist review	20% of claims, 15 minutes at ₹650 an hour = ₹32.50 per claim averaged	Assumption
Document AI and model cost	₹15 per claim	Assumption; list prices imply cents per file for basic extraction (UD)
Account management allocated	₹10,000 a month	Assumption
Delivery cost per hospital	**about ₹39,250 a month**	Calculated
Gross margin	**about 61%**	Calculated

Two sensitivities show where the model breaks. If the operator recovers 10% of leakage instead of 25%, revenue falls to about ₹67,500 a month and gross margin to about 42%. If human time doubles (40 minutes per claim and review on 40% of claims), cost rises to about ₹64,000 a month and gross margin at the base revenue falls to about 37%. Compute is a rounding error in both cases; recovery rate and human minutes decide everything. For comparison, secondary benchmarks put AI-native companies at roughly 50-60% gross margin against 75-90% for software, with practitioner guides (low authority) suggesting 35-45% in the first six months for AI services (SoftwareSeni; Kanopy Labs).

The scale implication is sobering. At these assumptions, 100 hospitals produce about ₹12 crore of annual revenue (roughly US$1.4M). A take rate of 0.7% applied to the whole ₹62,500 crore cashless pool is a ceiling of roughly ₹420 crore a year at full penetration; at 1.5% the ceiling is about ₹940 crore. The domestic claims desk alone is a solid business and a narrow venture case. It needs either a higher take rate, proven by results, or the expansion paths described later.

Go-to-market

Sell the audit, not the platform. The opening offer to a hospital's owner or finance head is a no-cost review of the last quarter's settlements that ends with a rupee figure for contestable deductions, followed by recovery on contingency. This follows Sequoia's advice to start where the buyer already experiences the work as an outsourced, rule-heavy task and the decision is a vendor swap, and Foundation Capital's warning that proofs of concept cost headcount hours (Sequoia; Foundation Capital). Concentrate on one city and the three or four payers and TPAs that dominate its claims, so the rules library and payer relationships compound. Channels worth testing are hospital associations (AHPI has made deductions its public cause), hospital-software vendors that lack a claims module, and chartered accountants who audit hospitals and see the receivables problem. Generic guidance, from low-authority sources, is that Indian buyers pay far less for software than Western ones but have an established habit of paying for outsourced work (upGrowth); that supports pricing against labour and recovered money. No source gives hospital sales-cycle length.

24-month roadmap

The milestones are targets I am proposing, not forecasts drawn from evidence.

Period	Build	Commercial milestone	Proof required to proceed
Months 0-2	Nothing beyond spreadsheets and document tooling	25-30 hospital interviews; 3-5 manual audits on real settlement files	Contestable deductions are material; at least 3 hospitals sign contingency terms
Months 3-6	Intake, extraction, reconciliation ledger, appeals workflow	5-10 design-partner hospitals in one city	Measured recovery rate and human minutes per claim; first money recovered
Months 7-12	Rules engine, gap finder, submission composer, portal channel layer; NHCX sandbox certification	25-40 hospitals; first live-desk takeovers	Clean-claim and deduction rates improve against each hospital's own baseline; gross margin above 40%
Months 13-18	Query desk automation, analytics for hospital management, government-scheme module pilot	About 100 hospitals; second city; financing partner signed	Review rate falling quarter on quarter; payers accepting submissions without friction
Months 19-24	Multi-site controls for chains; second workflow pilot	150-200 hospitals including one chain; decision on US or payer-side expansion	Net revenue retention and payback that justify a larger raise

Moats

Four advantages can accumulate, and none is guaranteed. The first is the denial-reason dataset described above, which improves first-time-right rates and cannot be bought. The second is the learning loop from human corrections, which lowers cost per claim with volume. The third is workflow ownership: once the operator runs the desk and holds the reconciliation ledger, switching means rebuilding a function. The fourth is underwriting data for receivables financing. The honest weakness is that the submission channels are owned by organisations whose interests oppose yours, and NHCX, which would neutralise that, is not yet dependable.

Team

The founding group needs someone who has run a hospital insurance desk or a TPA's provider operations, since the work is mostly knowing how payers behave. Around that person: a claims operations lead, medical reviewers (doctors or certified coders), engineers for document AI and workflow, forward-deployed implementers who sit in hospitals during onboarding, and a security and compliance owner from day one. Indian wages make the human layer affordable. Aggregator data puts early-career claims processors at about ₹2-3 lakh a year and medical coders at a ₹3.6 lakh median (Payscale; Hyring). These are self-reported figures, and no authoritative wage survey was found.

Risks and mitigations

Risk	Why it matters	Mitigation
Hospitals will not pay enough	No published willingness-to-pay data; Indian software prices are low	Contingency-only entry; validate price before building; add financing revenue
Recoverable share is small	Much of the disallowed amount is legitimately non-payable	Manual audits first; price on measured recovery; shift value to prevention and faster discharge
Payers or rails block third-party operation	IHX and Medi Assist control the portals and build their own AI	Operate under the hospital's named users and written authority; pursue NHCX certification; avoid adversarial positioning
Exception rates stay high	Human minutes decide margin	Narrow scope by payer and specialty at first; instrument every intervention
Government-scheme dues	Liquidity problem that software cannot solve	Exclude from early pricing promises; treat as a later module
Accusation of claim inflation	Fraud framework reaches vendors; reputational end-state	Never touch clinical content; full audit trail; clinician sign-off
Data breach of health records	Penalties up to ₹250 crore; hospital pushes liability down by contract	India-region hosting, encryption, logging, one-year log retention, breach process built to the 2027 standard
Funded competitor moves first	Care.fi already bundles software and financing	City-level density and measurable outcomes; partner with lenders early
Insurance cover excludes AI errors	Insurers are adding AI exclusions to professional liability policies	Obtain technology errors-and-omissions and cyber cover with AI wording confirmed in writing

Expansion path

The first step outward is receivables financing through lending partners, using the ledger as underwriting data; Care.fi's model shows the pairing is fundable, and it claims funds reach hospitals within 3-5 days of discharge (a vendor claim) (CXOToday). The second is government schemes, where PM-JAY alone has 15,733 private empanelled hospitals (Millennium Post). The third is patients' reimbursement claims filed from the hospital at discharge. The fourth, and the largest pool, is US revenue-cycle work delivered from India: Zinnov puts US revenue-cycle spend above US$110bn with over 60% of the global workforce in India (Zinnov). That market is being repriced from 5-9% to 1-1.5% of collections by a competitor built on an Indian delivery firm, so it is an opportunity with a shrinking price. Payer-side products for mid-sized TPAs are possible later but create a conflict of interest with hospital customers and should be a deliberate choice.

What global AI-services companies teach about the model

By 2026 "sell the work, not the tool" is consensus among leading funds, justified by labour and services budgets several times larger than software budgets. Sequoia cites six dollars of services for every dollar of software (Sequoia). These are spending pools that show order of magnitude; they are not capturable revenue. Five lessons transfer to this product.

Sell the outcome to the party that carries the pain. Sequoia distinguishes a copilot sold to a professional from an autopilot sold to the end buyer, and notes that when you sell the work, improvements in the underlying models widen your margin, whereas a tool-seller is racing the model. Harper abandoned selling tools to insurance brokers and became the broker (TechCrunch).

Someone accountable owns the output. Crosby is itself a law firm with licensed attorneys (Law360), Harper holds the licence, and EvenUp and Altair keep professionals on review. No source describes a company shipping unreviewed regulated output.

Integration is the product. Foundation Capital's review of the first year concluded that integration is the product surface and that outcome pricing is hard to standardise because results depend on the customer's own processes (Foundation Capital). That is why this report insists on a baseline audit before any success fee.

Price against labour and money at stake. Hesper AI, a three-person 2026 startup investigating property and casualty claims, benchmarks itself against US$1,500-2,500 per outsourced investigation (Y Combinator).

Build, do not buy, at the start. Emergence argues that acquired services revenue often does not stick and that the best AI-native firms are built organically (Emergence); evidence for roll-up margin gains is almost entirely investor-reported.

The comparables need caveats. EvenUp's valuation above US$2bn is confirmed by the company (EvenUp), but its revenue figures are conflicting third-party estimates, and no verified gross margin, review ratio or unit price exists for any company named here. Altair Health is a US business whose funding and pricing are undisclosed and whose website metrics may be illustrative. Hesper AI's funding is reported inconsistently. Two Indian names sometimes offered as precedents are not: Hulp is a consumer concierge service for households (Inc42), and Adalat AI is a philanthropy-funded non-profit providing transcription to courts (Bar & Bench). Neither is a business selling a regulated workflow outcome to enterprises, and they show only that mixed AI-and-human delivery and public-sector adoption exist in India.

Indian rules decide who signs and who is liable

The Digital Personal Data Protection Rules were notified in November 2025. Consent-manager registration opens on 13 November 2026, and the substantive duties (notice and consent, fiduciary obligations, rights, penalties) bind from 13 May 2027, six months from now (AZB & Partners). Penalties reach ₹250 crore for failure to take reasonable security safeguards (via the PIB overview; verify against the Gazette) (PIB). The Act has no separate sensitive-data class, but the 2011 rules that treat health and financial data as sensitive stay in force until May 2027, so both regimes apply meanwhile (King Stubb & Kasiva). For this product the hospital is the data fiduciary and remains responsible for its processors, so it will push security terms, audit rights and breach-notice clauses into the contract. There is no general localisation rule, though India-region hosting is the practical default. AI itself is governed by voluntary guidelines issued in November 2025, which state that a separate AI law is not needed at this stage (Mondaq).

Sector outsourcing rules are the reason the insurer-side and banking candidates score poorly on liability. RBI's 2023 direction on IT outsourcing requires that outsourcing not impede the regulated entity's oversight and that providers report cyber incidents fast enough for the entity to notify RBI within six hours (Vinod Kothari); 2025 directions reportedly add audit rights and direct supervisory access to the provider, but the final texts were not read. IRDAI's position on what claims work an insurer may outsource is unverified in its current form. A vendor serving the hospital sits outside both regimes on the evidence gathered, which is a structural advantage of the recommended product and one to confirm with counsel.

Professional-practice rules shape the other candidates. In tax, the list of who may sign and appear is closed. In law, only enrolled advocates may practise, fee-sharing with non-advocates is barred, and the Bar Council has told Parliament that using AI is no defence to misconduct (Bar & Bench). The notes retrieved no source on restrictions under the Chartered Accountants Act, so the tax runner-up carries an unexamined professional-rules question.

Eight things to validate in the first 30-60 days

#	Question	How to test it	Result that supports going ahead
1	How much deduction and denial value is actually contestable?	Manual audit of 3-5 hospitals' last quarter of settlements	A material share is recoverable within payer appeal windows
2	Will hospitals pay, and on what basis?	Offer contingency and per-claim terms to 25-30 owners and finance heads	Signed terms at or above the model's fee levels
3	How many human minutes does a claim take?	Time every step in the manual audits and one live desk	Consistent with the model's 20 minutes plus review, or a clear path there
4	Can a third party operate payer portals on the hospital's behalf?	Read portal terms; ask two TPAs and IHX directly	No prohibition on authorised named users
5	Is NHCX usable today?	Sandbox registration; ask hospitals and insurers what share of claims flows through it	Enough live payers to justify building the channel in year one
6	Does any law reserve this work or treat the vendor as an insurer's outsourcer?	Written opinion from a health-insurance regulatory lawyer	Confirmation that a hospital-side vendor is outside IRDAI outsourcing rules
7	What do Care.fi, ClaimBuddy and outsourced billing firms charge?	Mystery-shop through friendly hospitals; reference calls	A price umbrella the model fits under
8	Can the firm buy liability cover that includes AI output?	Quotes from two brokers for technology errors-and-omissions and cyber	Affirmative AI wording available at tolerable premium

Alongside these, re-check three numbers against primary documents before they go into a pitch: the FY25 value of disallowed and repudiated claims in the IRDAI annual report, the cashless share by number of claims, and whether the FY26 annual report has been published.

Conclusion

The six sectors differ less in how much money flows through them than in who is allowed to finish the job. In law, tax, banking and insurer-side claims, a statute or a regulator hands the last step to a licensed person or a regulated entity, which turns an "AI operations company" into either a staffing arrangement around professionals or a tool vendor to them. The hospital claims desk is the one workflow where the customer is the claimant, the work is already clerical, and the measure of success is money arriving in a bank account. That is the real reason it wins, more than its market size.

The same analysis shows the limit. At plausible Indian prices the domestic desk is a business worth tens of crores of revenue, and the notes contain no evidence that hospitals will pay even that. The founder should treat the first two months as a test of one number, the share of withheld money that can be recovered, and should be prepared to move to the tax runner-up or to US revenue-cycle work if that number is small. If it is large, the reconciliation ledger built in the wedge becomes the base for financing, scheme claims and export work, and the company's value will come from those layers more than from the per-claim fee.